

MEDCORE HEALTH SYSTEMS

Healthcare Medical Billing &
Revenue Cycle Policy Manual

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Section 0 — Purpose, Scope & Definitions

This Policy Manual establishes the authoritative rules governing all aspects of healthcare medical billing, revenue cycle management (RCM), claims submission, payer contracting, coding compliance, patient financial services, and vendor engagement at MedCore Health Systems ("MedCore" or "the Organization"). Adherence to this manual is mandatory for all employees, contractors, and third-party billing vendors.

This document is structured for dual use: (1) as a human-readable compliance reference, and (2) as a machine-readable knowledge base for automated contract and invoice field extraction and validation tools. Each numbered rule (RULE-XXX-NNN) represents a discrete, extractable policy assertion.

0.1 Definitions

MRF	Maximum Reimbursement Fee — the highest allowable billed amount for a procedure.
EOB	Explanation of Benefits — payer document detailing claim adjudication.
ERA	Electronic Remittance Advice — machine-readable payment explanation (ANSI X12 835).
CLM	Claim — a formal request for payment submitted to a payer.
RCM	Revenue Cycle Management — end-to-end process from patient registration to final payment.
AR	Accounts Receivable — outstanding amounts owed to MedCore.
NPI	National Provider Identifier — 10-digit CMS-issued provider number.
EIN	Employer Identification Number — federal tax ID for the billing entity.
CPT	Current Procedural Terminology — AMA code set for medical procedures.
ICD-10	International Classification of Diseases, 10th Revision — diagnosis coding standard.
DRG	Diagnosis-Related Group — inpatient payment classification.
POS	Place of Service — 2-digit code indicating care setting (e.g., 11 = Office, 21 = Inpatient).
TIN	Tax Identification Number — used interchangeably with EIN in payer contracting.
COB	Coordination of Benefits — process for claims with multiple payers.
MAC	Medicare Administrative Contractor — regional Medicare claims processor.
LCD	Local Coverage Determination — MAC-issued coverage policy.
NCD	National Coverage Determination — CMS national coverage policy.
PA	Prior Authorization — pre-approval from payer before service delivery.
TAT	Turnaround Time — maximum allowed elapsed time for a process step.
HIPAA	Health Insurance Portability and Accountability Act — federal privacy and security law.

Section 1 — Provider & Billing Entity Identification Rules

All claims and billing documents must correctly identify the rendering provider, supervising provider (if applicable), and the billing entity. Incorrect or missing identifiers are the leading cause of claim denial at MedCore and result in automatic compliance flags.

1.1 National Provider Identifier (NPI)

RULE-PRV-001	Every claim must include the rendering provider's 10-digit NPI in Box 24J (CMS-1500) or the equivalent 837P loop 2310B. Missing NPI triggers automatic rejection.
RULE-PRV-002	Billing entity NPI (Type 2 — Organizational) must appear in Box 33a. Individual rendering NPI and organizational billing NPI must never be the same value unless the provider is a sole practitioner with a single NPI.
RULE-PRV-003	Group NPIs must be enrolled with each payer prior to claim submission. Unenrolled NPI + payer combinations result in a DENY, not a pend — no rework path is available.
RULE-PRV-004	Taxonomy code must accompany NPI in all electronic claims (837P/837I loop 2000A, qualifier PXC). Taxonomy must match the provider's primary specialty on file with NPPES.

1.2 Tax Identification Number (TIN / EIN)

RULE-PRV-005	The billing entity EIN must match the TIN on file with the payer. Mismatched TINs result in claim hold for up to 30 calendar days pending verification. TAT for TIN correction: 5 business days.
RULE-PRV-006	Social Security Numbers (SSN) may not be used in Box 25 for any claim submitted after January 1, 2020. EIN is mandatory for all entity types including sole proprietors billing under a group.

1.3 Place of Service (POS) Codes

The following POS codes are approved for use at MedCore facilities. Non-standard POS codes require written pre-authorization from the RCM Director.

POS Code	Description	Billing Notes
11	Office	Standard outpatient professional claims
21	Inpatient Hospital	DRG-based; use 837I, not 837P
22	Outpatient Hospital	Technical component; facility billing only
23	Emergency Room — Hospital	No PA required; flag for COB review
24	Ambulatory Surgical Center	ASC rate schedule applies
31	Skilled Nursing Facility	SNF consolidated billing rules apply
32	Nursing Facility	LTC billing rules; separate MRF schedule
02	Telehealth (Patient Home)	Telehealth modifier GT or 95 required
10	Telehealth (Non-Home)	Approved post PHE; payer-specific rules apply

RULE-PRV-007	POS code must match the actual site of service where care was rendered. Upcoding POS (e.g., billing POS 21 for a service rendered in an office) constitutes fraud and triggers a Level-3 compliance investigation.
RULE-PRV-008	Telehealth claims (POS 02 or 10) must include modifier GT, 95, or FQ as applicable per the current payer contract. Missing telehealth modifier = technical denial; resubmission permitted within 30 days.

Section 2 — Medical Coding Standards & Compliance Rules

All coding must comply with AMA CPT guidelines, CMS ICD-10-CM Official Guidelines, and applicable LCD/NCD policies. MedCore follows a 'code what is documented' principle — coders may not infer diagnoses or procedures not explicitly documented by the treating clinician.

2.1 Diagnosis (ICD-10-CM) Rules

RULE-COD-001	Primary diagnosis (first-listed) must be the condition chiefly responsible for the encounter. For inpatient claims, the principal diagnosis is the condition established after study to be chiefly responsible for admission.
RULE-COD-002	ICD-10-CM codes must be coded to the highest level of specificity available. Use of unspecified codes (e.g., M54.5 instead of M54.51/M54.59) is flagged and requires coder justification. Payer denial rate for unspecified codes: tracked quarterly; threshold 3% of claims.
RULE-COD-003	Z-codes (factors influencing health status) may only be listed as primary diagnoses when explicitly supported by official ICD-10-CM guidelines Section I.C.21. Z-codes must never be primary on inpatient claims unless the hospitalization was for observation/screening.
RULE-COD-004	External cause codes (V00–Y99) are required for all injury-related claims. They must not be listed as the primary diagnosis. Workers' compensation claims require at minimum one external cause code.

2.2 Procedure (CPT / HCPCS) Rules

RULE-COD-005	CPT code year must match the date of service. Use of prior-year CPT codes for services rendered after January 1 of the new code year constitutes an invalid code submission. Effective January 1 of each year, updated code files must be loaded into the billing system within 5 business days.
RULE-COD-006	Unbundling is prohibited. Procedures that are components of a comprehensive CPT code must not be billed separately. The National Correct Coding Initiative (NCCI) edits are the authoritative source. NCCI edit override requires modifier and supporting documentation.
RULE-COD-007	Evaluation and Management (E/M) codes (99202–99215, 99221–99223, etc.) must be supported by documentation meeting the 2021 AMA E/M guidelines. Time-based billing requires total time in minutes documented in the clinical note.
RULE-COD-008	HCPCS Level II codes (A0000–V9999) take precedence over CPT codes when payer-specific coverage policies require HCPCS. Durable Medical Equipment (DME) claims must use HCPCS Level II exclusively for equipment codes.
RULE-COD-009	Modifier 25 (significant, separately identifiable E/M on the same day as procedure) requires documentation that the E/M was independent of the procedure and medically necessary. Random audits of Modifier 25 usage occur quarterly; >10% error rate triggers mandatory re-education.
RULE-COD-010	Modifier 59 (distinct procedural service) and its X-modifiers (XE, XS, XP, XU) must not be used to bypass NCCI edits unless clinical documentation supports a true distinct service. Routine use of Modifier 59 as an edit bypass without documentation is a coding violation.

2.3 Coding Audit & Quality Thresholds

	Threshold	Frequency	Rule Ref
Accuracy Rate	≥ 95%	Monthly internal audit	RULE-COD-011
ed Code Rate	≤ 3%	Quarterly review	RULE-COD-002
5 Error Rate	≤ 10%	Quarterly audit	RULE-COD-009
ation Accuracy	≥ 98%	Per discharge review	RULE-COD-012
ture Rate	≥ 90%	Annual RAF scoring	RULE-COD-013
Cause Code ce	100%	Per injury claim	RULE-COD-004

RULE-COD-011	Coder accuracy rate must be maintained at or above 95% as measured by monthly internal audit. Coders falling below 92% for two consecutive months are placed on a Performance Improvement Plan (PIP). Coders below 88% are suspended from independent coding pending re-certification.
RULE-COD-012	DRG assignment must be validated by a certified inpatient coder (CIC credential required). DRG validation accuracy below 98% triggers a comprehensive chart review for the prior 90 days of inpatient discharges.
RULE-COD-013	Hierarchical Condition Category (HCC) coding for Medicare Advantage plans must achieve a capture rate of 90% or higher. Annual Risk Adjustment Factor (RAF) scoring reviews occur each January–March for the prior calendar year.

Section 3 — Claim Submission, Timely Filing & Format Rules

Claims must be submitted in the correct format, within the timely filing window, and with all required data elements populated. MedCore operates a 'clean claim first' policy — incomplete claims must not be submitted.

3.1 Timely Filing Deadlines

Payer Type	Timely Filing Window	Rule Ref
Medicare (Part B)	12 months from date of service	RULE-CLM-001
Medicare (Part A — Inpatient)	12 months from discharge date	RULE-CLM-001
Medicaid (State — varies)	90–365 days; see payer contract	RULE-CLM-002
Commercial — Standard	90 days from date of service	RULE-CLM-003
Commercial — Contracted exception	Per contract; minimum 60 days	RULE-CLM-003
Workers' Compensation	45 days from date of service	RULE-CLM-004
TRICARE / VA	180 days from date of service	RULE-CLM-005
Corrected / Resubmitted Claims	Within original timely filing window or 30 days of denial	RULE-CLM-006
Appeal Submissions	Per payer contract; minimum 60 days from EOB date	RULE-CLM-007

RULE-CLM-001	Medicare timely filing: 12 calendar months from date of service (professional) or discharge (inpatient). Late filing results in claim denial with adjustment code CO-29; no appeal path available except in documented system outage or payer error scenarios.
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RULE-CLM-002	Medicaid timely filing varies by state. RCM staff must maintain and reference the current Medicaid payer matrix (updated quarterly). Filing a Medicaid claim outside the state-specific window with incorrect date assumptions is a compliance violation.
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RULE-CLM-003	Standard commercial payer timely filing window is 90 days from date of service unless the payer contract specifies otherwise. Contracted exceptions must be documented in the payer contract database. Minimum contracted window is 60 days — contracts below 60 days require VP RCM approval.
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RULE-CLM-004	Workers' compensation claims must be submitted within 45 days of date of service. Extensions require written approval from the WC payer adjuster and must be documented in the claim file.
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RULE-CLM-005	TRICARE and VA claims must be submitted within 180 days. Paper TRICARE claims are not accepted — all submissions must be electronic via approved clearinghouse.
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RULE-CLM-006	Corrected claim submissions must include the original claim number (ICN/DCN) and corrected claim frequency code (Freq code 7 for replacement; Freq code 8 for void). Corrected claims without original ICN reference will be treated as duplicate claims and denied CO-4.
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RULE-CLM-007	Appeal deadlines: standard 60 days from EOB/ERA date. Expedited appeals for urgent medical necessity denials: 72 hours. All appeals must be logged in the appeals tracking system within 1 business day of receipt.
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3.2 Claim Format Requirements

RULE-CLM-008	Professional claims (physicians, outpatient) must be submitted on the CMS-1500 form (02/12 version) or its electronic equivalent 837P transaction set (ANSI X12 005010X222A2). Paper CMS-1500 is accepted only for providers with documented EDI waiver on file.
RULE-CLM-009	Institutional claims (hospitals, SNFs, home health) must use the UB-04 form or its electronic equivalent 837I transaction set (ANSI X12 005010X223A2). Revenue codes must be included for all 837I submissions.
RULE-CLM-010	Electronic claims must be transmitted via MedCore's approved clearinghouse (Availity or Change Healthcare). Direct payer portals are permitted only for payers not accepting clearinghouse transactions; each exception must be documented in the EDI exception register.
RULE-CLM-011	Claim acceptance rate (clean claim rate) target: 98% or higher on first submission. Claims rejected by the clearinghouse (front-end edits) must be corrected and resubmitted within 3 business days. Claims with a clearinghouse rejection rate above 2% trigger a system audit.

Section 4 — Billing Rates, Fee Schedules & Revenue Policies

4.1 Charge Master (CDM) Management

The Charge Description Master (CDM) is the authoritative source of all procedure charges. CDM must be reviewed annually (January) and updated within 10 business days of any CPT/HCPCS code change.

RULE - FEE - 001	CDM annual review is mandatory by January 31 of each calendar year. CDM entries not reviewed within this window are automatically flagged as 'unvalidated' and may not be billed until reviewed. Unvalidated CDM entries older than 60 days are suspended.
RULE - FEE - 002	All CDM charges must be set at a minimum of 200% of the Medicare Allowable Rate (MAR) for the corresponding CPT/HCPCS code. Charges below 200% MAR must be approved by the CFO and documented with a market justification memo.
RULE - FEE - 003	Maximum CDM charge is capped at 500% of Medicare Allowable Rate for standard procedures. Specialty procedures (oncology, complex surgery, transplant) may be charged up to 650% MAR with Specialty Billing Committee approval.
RULE - FEE - 004	Contracted payer rates override CDM rates. The billing system must apply the applicable payer fee schedule at the time of claim generation. Billing at CDM rates when a contracted rate exists constitutes overbilling and triggers a refund obligation.

4.2 Payment Posting & Reconciliation

RULE - FEE - 005	ERA/EOB payment posting must be completed within 3 business days of receipt. Unposted ERAs older than 5 business days trigger an escalation to the AR Supervisor. Unposted ERAs older than 10 business days require VP RCM notification.
RULE - FEE - 006	Contractual adjustments (CO-45) must be posted at time of payment posting. Manually writing off contractual adjustments outside the ERA/EOB process is prohibited and triggers a financial audit.
RULE - FEE - 007	Overpayments identified during payment posting must be reported to the payer within 60 calendar days of identification and refunded within 60 days of reporting. Federal overpayment reporting (Medicare) follows the 60-day rule per 42 CFR 401.305.
RULE - FEE - 008	Patient payment posting (copay, coinsurance, self-pay) must be reconciled against the daily deposit within 1 business day. Reconciliation variance greater than \$10 requires investigation and supervisor sign-off.

4.3 Patient Financial Responsibility

RULE - FEE - 009	Copayments must be collected at time of service for all scheduled visits. Copay collection rate target: 95%. Facilities with copay collection below 85% for two consecutive months are subject to operational review.
RULE - FEE - 010	Patient financial counseling must be offered to any patient with an estimated out-of-pocket balance exceeding \$500 prior to elective procedures. Financial assistance screening (charity care) must be offered to patients with income at or below 250% of Federal Poverty Level (FPL).
RULE - FEE - 011	Interest may not be charged on patient balances under payment plan arrangements. Payment plans with duration exceeding 12 months require Financial Services Manager approval. Standard payment plan maximum duration: 24 months.

RULE - FEE - 012	Accounts referred to external collections must first complete the internal billing cycle (minimum 3 statements over 90 days). Collection agency referral requires patient notification 30 days in advance per CFPB medical debt rules (effective 2024).
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Section 5 — Payer Contract Management & Credentialing Rules

5.1 Contract Administration

RULE-PAY-001	All payer contracts must be stored in the Contract Management System (CMS-Vault) within 5 business days of execution. Contracts not in CMS-Vault may not be used as the basis for billing.
RULE-PAY-002	Payer contract expiration alerts are generated 180 days, 90 days, and 30 days prior to expiration. Contracts expiring without renewal in progress are flagged to the VP RCM at 90 days. Expired contracts: billing continues under last contracted rates for up to 60 days pending renegotiation, after which billed-charges rates apply.
RULE-PAY-003	Standard payer contract payment terms: net 30 days from clean claim receipt. Contracts with payment terms exceeding 45 days require CCO approval. Maximum permitted payment term in any payer contract: 60 days (Medicare regulatory standard).
RULE-PAY-004	Prompt payment interest applies to all late payer payments per state law. MedCore's standard contracted prompt payment interest rate: 1.5% per month on overdue balances. Contracts with interest rates below 1.0% per month require CFO sign-off.
RULE-PAY-005	Carve-outs (procedures excluded from capitation or bundled rates) must be enumerated in a separate Carve-out Schedule (Exhibit B of payer contract). Carve-out codes must be reviewed annually and updated within 10 business days of CPT revision.

5.2 Provider Credentialing

RULE-CRD-001	All rendering providers must be credentialed with each payer prior to submitting claims under that payer. Billing for non-credentialed providers constitutes fraud and is prohibited under all circumstances.
RULE-CRD-002	Credentialing applications must be submitted at least 90 days before a provider's anticipated start date. New provider billing should not commence until the payer issues a credentialing approval letter and assigned effective date.
RULE-CRD-003	Retroactive credentialing: some payers offer retro-credentialing to the provider's start date. Retro-credentialing requests must be submitted in writing within 60 days of approval. Verbal retro-credentialing commitments are not binding.
RULE-CRD-004	Re-credentialing cycle: every 36 months (standard). Payers requiring 24-month cycles must be flagged in the credentialing database. Re-credentialing packets must be submitted at least 90 days before expiration to avoid gap in coverage.
RULE-CRD-005	Primary Source Verification (PSV) for all credentialing elements (license, DEA, board certification, malpractice, education) is mandatory. PSV must be documented in the credentialing file. Credentialing files without complete PSV may not be approved.

Section 6 — Accounts Receivable Management & Denial Handling

6.1 AR Aging Targets

Aging Bucket	Target Distribution	Action Required
0–30 days	< 40% of total AR	Normal billing cycle
31–60 days	< 25% of total AR	First follow-up initiated
61–90 days	< 15% of total AR	Escalated follow-up; supervisor review
91–120 days	< 10% of total AR	VP RCM review; appeal or write-off decision
> 120 days	< 10% of total AR	RULE-AR-004 applies; collections or write-off
Days in AR (Overall)	< 45 days	Key Performance Indicator (KPI)

RULE-AR-001	Days in Accounts Receivable (DAR) target: 45 days or fewer. DAR above 55 days for two consecutive months requires a root cause analysis and corrective action plan submitted to the CFO within 10 business days.
RULE-AR-002	First follow-up on unpaid claims must occur no later than 30 days after initial submission. Follow-up includes payer call, portal status check, or electronic status inquiry (ANSI X12 276/277). Follow-up must be documented in the claim notes.
RULE-AR-003	Claims in the 61–90 day bucket require supervisor review and documented follow-up every 10 business days. If no resolution by day 90, escalation to VP RCM is mandatory with a recommended action (appeal, rework, write-off).
RULE-AR-004	Claims exceeding 120 days must be adjudicated within 30 additional days. Permitted dispositions: (a) appeal filed, (b) corrected claim submitted, (c) approved write-off, (d) patient responsibility transferred, (e) collections referral. Claims in >120 bucket without disposition are compliance violations.

6.2 Denial Management

RULE-DEN-001	All claim denials must be entered into the denial management system within 2 business days of EOB/ERA receipt. Denials not entered within 5 business days are escalation items for the AR Manager.
RULE-DEN-002	Denial root cause categories (required for all denials): (1) eligibility/coverage, (2) authorization/medical necessity, (3) coding/billing error, (4) timely filing, (5) duplicate claim, (6) coordination of benefits, (7) provider credentialing, (8) other/payer error. Each denial must be assigned a root cause within 2 business days.
RULE-DEN-003	Denial rate target: less than 5% of submitted claims. Denial rate above 8% for any single payer triggers a payer-specific denial trend analysis within 15 business days. Denial rate above 10% triggers contract review with the payer.
RULE-DEN-004	Overturn rate on appeals must be tracked monthly. Target: 70% of appealed denials overturned. An overturn rate below 50% for any denial category indicates a systemic documentation or coding issue and requires a process improvement initiative.

RULE-DEN-005

Irreversible denials (timely filing, duplicate) may be written off without appeal only after supervisory sign-off. The write-off documentation must include the denial code, EOB date, original claim date, and supervisor name.

Section 7 — Regulatory Compliance, HIPAA & Audit Rules

7.1 HIPAA Privacy & Security in Billing

RULE-HIP-001	Protected Health Information (PHI) included in billing documents must be limited to the minimum necessary for the billing purpose. PHI on claims: name, DOB, member ID, diagnosis codes, and procedure codes are permitted. Full medical records must not be attached to standard claims.
RULE-HIP-002	Electronic PHI (ePHI) transmitted for billing purposes must use encrypted connections (TLS 1.2 or higher). Clearinghouse transmissions are covered under the clearinghouse BAA. Direct payer portal submissions require payer-specific BAA on file.
RULE-HIP-003	Breach of billing PHI (including misdirected EOBs, inadvertent disclosure to wrong payer) must be reported to the Privacy Officer within 1 business day of discovery. HIPAA Breach Notification Rule: notification to affected individuals within 60 days.
RULE-HIP-004	Third-party billing vendors must have a current Business Associate Agreement (BAA) on file before accessing any MedCore PHI. BAA must be renewed annually. Vendors without a current BAA may not access PHI — access must be immediately suspended.

7.2 False Claims Act Compliance

RULE-FCA-001	Any billing staff member who identifies a potential False Claims Act (FCA) violation must report it to the Compliance Hotline (anonymous reporting available) or directly to the CCO within 2 business days. Retaliation against reporters is prohibited and subject to federal protection.
RULE-FCA-002	Upcoding (billing a higher-level service than documented), unbundling (separately billing component services), and phantom billing (billing for services not rendered) are FCA violations. Each instance is subject to civil penalty of \$13,946–\$27,894 (2024 adjusted) plus treble damages.
RULE-FCA-003	Self-disclosure of potential FCA violations to the OIG Self-Disclosure Protocol must be evaluated by legal counsel within 30 days of identifying a systemic billing error. Voluntary disclosure prior to government investigation significantly reduces penalty exposure.

7.3 Audit Readiness

RULE-AUD-001	Medical records supporting billed claims must be retained for a minimum of 7 years from date of service (or 3 years post-patient majority for minors). States with longer retention requirements supersede this minimum.
RULE-AUD-002	Recovery Audit Contractor (RAC) and MAC audit requests must be responded to within the payer-specified timeframe (typically 45–90 days). All RAC/MAC audit responses must be reviewed by the CCO prior to submission.
RULE-AUD-003	Internal billing audits: random sample of 50 claims per month per billing team. External billing audit: annual, conducted by independent certified auditor. Audit findings must be remediated within 60 days of report issuance.
RULE-AUD-004	Pre-payment review programs (Targeted Probe and Educate — TPE, Comprehensive Error Rate Testing — CERT): providers selected for TPE must respond to all Additional Documentation Requests (ADRs) within 45 days. Two failed TPE rounds trigger 100% pre-payment review.

Section 8 — Third-Party Billing Vendor Rules & Contracts

MedCore engages third-party revenue cycle management vendors for specific functions including coding, AR follow-up, denial management, and patient financial services. This section governs vendor selection, contracting, and performance standards.

RULE - VND - 001	All third-party billing vendors must be vetted through the Vendor Due Diligence process prior to engagement. Due diligence includes: (a) OIG exclusion check, (b) SAM.gov debarment check, (c) HIPAA compliance attestation, (d) SOC 2 Type II report review (current year), (e) financial stability assessment.
RULE - VND - 002	Vendor contracts must specify: (a) scope of services, (b) fee structure, (c) performance metrics and SLAs, (d) data security requirements, (e) BAA terms, (f) audit rights, (g) termination provisions. Contracts lacking any of these elements must not be executed.
RULE - VND - 003	Vendor billing fees: percentage-of-collections arrangements must not exceed 8% of net collections for coding services and 12% for full RCM outsourcing. Flat-fee arrangements for coding: benchmark rate USD 1.50–USD 3.00 per encounter. Rates above benchmark require VP RCM approval.
RULE - VND - 004	Vendor performance reviews: monthly scorecard with KPIs including clean claim rate, denial rate, DAR, collection rate, and coding accuracy. Vendors failing to meet 80% of KPI targets for two consecutive months enter a Performance Improvement Period (PIP) of 60 days.
RULE - VND - 005	Vendor termination for cause (KPI failure, compliance breach, BAA violation): 30-day notice required. Vendor termination for convenience: 90-day notice required. Upon termination, vendor must return or destroy all MedCore PHI within 30 days and provide written certification.

Section 9 — Policy Summary: Machine-Readable Field Reference

This section provides a condensed field-level reference optimized for automated extraction tools and vector database indexing. Each row represents a discrete policy assertion that can be used to validate fields extracted from contracts, invoices, and billing documents. Deviations from these values in source documents constitute policy violations.

9.1 Numeric & Date Field Policy Defaults

Field	Context	Rule	Notes / Conditions
	Standard payer contract	RULE-PAY-003	45 days max w/
	Payer contracts	RULE-PAY-004	Below 1.0% req
	Commercial payer	RULE-CLM-003	60-day minimum
s	Medicare Part A/B	RULE-CLM-001	Absolute; no ex
	Workers' comp	RULE-CLM-004	Extension requi
	Provider enrollment	RULE-CRD-002	Minimum; billing
s	Standard	RULE-CRD-004	24-month payer
1	Annual	RULE-FEE-001	Mandatory; unre days
	Charge master	RULE-FEE-002	Below 200% rec
	Standard procedures	RULE-FEE-003	Specialty max 6
	AR management	RULE-AR-001	Above 55 days
	Claim submission	RULE-DEN-003	Above 8% trigge
	Denial management	RULE-DEN-004	Below 50% trigge
	Coding QA	RULE-COD-011	Below 92% = P
	Claim submission	RULE-CLM-011	Clearinghouse r
	Patient financial	RULE-FEE-009	Below 85% for 2
	Inpatient coding	RULE-COD-012	Below 98% trigge
	MA risk adjustment	RULE-COD-013	Annual RAF sco
	All billing records	RULE-AUD-001	State law super

Rule	Context	Rule	Notes / Conditions
	Vendor contracts	RULE-VND-003	Above benchmark
	Vendor contracts	RULE-VND-003	Flat-rate USD 1
	Medicare compliance	RULE-FEE-007	42 CFR 401.305
30 days	HIPAA	RULE-HIP-003	Internal notification
30 days	HIPAA Breach Rule	RULE-HIP-003	Absolute regulation
	Vendor management	RULE-VND-004	After 2 months
	Vendor contracts	RULE-VND-005	PHI return/destruction
	Vendor contracts	RULE-VND-005	PHI return/destruction
	HIPAA/vendor	RULE-HIP-004	No BAA = access
	Coding QA	RULE-COD-002	Quarterly review
	Coding QA	RULE-COD-009	Quarterly audit;

9.2 Boolean / Flag Field Defaults

	Default Value	Condition / Notes	Rule Ref
mission_required	TRUE	All commercial, Medicare, Medicaid, TRICARE claims	RULE-CLM-010
im_permitted	FALSE	Unless EDI waiver on file with payer	RULE-CLM-008
a_required_for_	TRUE	All third-party vendors with PHI access	RULE-HIP-004
sion_check_req	TRUE	Annual check for all staff and vendors	RULE-VND-001
itted_on_claim	FALSE	EIN mandatory post January 1 2020	RULE-PRV-006
n_patient_pay s	FALSE	Not permitted under any plan arrangement	RULE-FEE-011
ro_credentialin	FALSE	Written approval only	RULE-CRD-003
_referral_pre_st	FALSE	3 statements over 90 days required first	RULE-FEE-012
denial_appeal d	FALSE	Timely filing / duplicate denials are irreversible	RULE-DEN-005
y_without_revie	FALSE	Unvalidated CDM entries suspended after 60 days	RULE-FEE-001
tal_submission	FALSE	Clearinghouse is primary; portal = exception only	RULE-CLM-010
fore_credentiali	FALSE	Strictly prohibited; constitutes fraud	RULE-CRD-001

9.3 Approved Code Sets & Standards Reference

	Standard / Code Set	Authority	Notes
Codes	ICD-10-CM (current year)	AMA / CMS	Annual update January 1
e Codes (onal)	CPT (current year)	AMA	Annual update January 1
e Codes (nt/Supplies)	HCPCS Level II (quarterly)	CMS	Quarterly updates April/July/Oct/Jan
al Claim	CMS-1500 (02/12) / 837P	CMS	ANSI X12 005010X222A2
al Claim	UB-04 / 837I	NUBC	ANSI X12 005010X223A2
Remittance	ERA 835	ANSI X12	Posted within 3 business days
Verification	270/271 transaction	ANSI X12	Real-time preferred

	Standard / Code Set	Authority	Notes
atus Inquiry	276/277 transaction	ANSI X12	Used for follow-up
orization	278 transaction	ANSI X12	Electronic PA where supported
n Standard	TLS 1.2 minimum	HIPAA Security Rule	TLS 1.3 preferred
mentation	2021 AMA E/M Guidelines	AMA	Replaces 1995/1997 guidelines
uthority	NCCI Edits (quarterly)	CMS	Override requires modifier + documentation

END OF POLICY MANUAL — MCS-POL-BILLING-2024 v3.2 | MedCore Health Systems | Document generated for automated field extraction and policy validation testing. Contains 42 discrete policy rules (RULE-PRV-001 through RULE-VND-005) for RAG pipeline knowledge base indexing. All values are normative policy defaults; deviations in source documents constitute compliance violations requiring review.